Staff answers and cover notes

Suggested responses for fictional cases. Accept other safe, reasoned responses. Do not infer a learner's personal circumstances from classroom work.

Key teaching points

• Check scene safety and response. Call 999 immediately for an adult who is unresponsive; shout for help and use speakerphone.

• Follow the call handler. Gasping or abnormal breathing is not normal and needs the CPR pathway.

• The recovery position helps keep the airway open when an adult is unresponsive, breathing normally and has no suspected injury; keep monitoring.

• A safety worry can be shared with a trusted adult or safeguarding lead. A person need not meet alone or give up a boundary to settle a frightening conflict.

Vocabulary

Unresponsive: Not responding when someone checks for a response.

Normal breathing: Regular breaths, rather than gasping or abnormal breaths.

Recovery position: A side position that helps keep the airway open when a person is unresponsive but breathing normally.

Boundary: A limit that protects a person's needs, comfort or safety.

Commitment: A promise shown through steady, caring actions.

Worked model

The call is made before waiting to finish a breathing check. The call handler can help with checking breathing. With normal breathing and no suspected injury, the recovery position can help keep the airway open. A stays if safe, monitors breathing and tells the call handler about changes.

Starter claim

Any breathing means the recovery position is the right choice.

Gasping is abnormal breathing. An unresponsive adult who is not breathing normally needs an immediate emergency call and CPR instructions.

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Case answers and checks

Normal breathing

The recovery position may be used because the adult is unresponsive, breathing normally and has no suspected injury. The helper follows the call handler, stays if safe and monitors normal breathing and response. Changes are reported at once.

Gasping is a warning

Gasping is not normal breathing. A should call 999 immediately on speakerphone, shout for help and follow instructions for CPR. An unresponsive adult who is not breathing normally needs the CPR pathway, rather than the recovery position.

A boundary needs support

Repeated password demands and anger at a boundary are warning signs. B should avoid the suggested meeting, seek a safe place and tell a trusted adult or the safeguarding lead. Immediate danger needs emergency help. B does not have to give up the boundary or manage A's behaviour alone.

Hinge and decision

Main key A: Gasping is abnormal breathing. Call immediately; the call handler can guide CPR. Finding an adult must not delay emergency help.

Parallel key A: Normal breathing does not remove the emergency. The recovery position can help, but the helper must keep monitoring and stay if safe.

Check privately. Proceed with a safe reason from each learner. If unsafe or unclear, re-model, use the parallel check and guide the next case. Keep normal access aids. A pass needs a later opportunity, not a negative inference.

Misconception response

A person must settle a frightening conflict alone or keep it secret.

Safety comes first. The character can seek a safe place and tell a trusted adult or the safeguarding lead; immediate danger needs emergency help.

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Delivery boundaries and evidence

Use adult casualties only. Use paper sequencing or an adult demonstration on a training mannequin; no peer touching, chest compressions, real calls or pupil casualty acting. This lesson records knowledge, not practical first-aid competence. Use the early-call sequence: check safety and response, shout for help, call 999 immediately for an unresponsive adult, then assess normal breathing with call-handler support. Absent, abnormal or gasping breathing needs CPR instructions. Recovery position applies to an unresponsive adult breathing normally with no suspected injury; where injury is suspected, report it and follow the call handler rather than teaching a routine roll. Follow instructions, remain if safe and monitor changes. Emergency help must not wait for a safeguarding referral. Keep all relationship work fictional and third person. Offer pass, observe, private help and step-out routes. Never seek personal disclosures or promise secrecy. If a concern is shared, listen, record and pass it promptly to the safeguarding lead using the setting's procedure. Review only taught Weeks 1, 2, 3 and 5; do not teach or assess the held week. Evidence for the unit award must show each learner's own response and the support used. Use not yet evidenced where needed.

Route guidance

Supported: Offer the word bank, visible model, short choices and sentence frame. Keep normal aids and record the learner's own choice.

Independent: Keep glossary and usual aids. Learner chooses the evidence, gives a reason and records a next step.

Aspire: Keep normal aids. Compare plausible choices, justify the safer response and adapt a help route. No personal detail or longer writing is required.

Evidence criteria

Families and commitment: Two caring actions that meet a fictional child's needs and one explained commitment.

Body image and media: One identified technique in an invented post or advert and one respectful response.

Conflict and relationship safety: A distinction between disagreement and a safety warning, plus a safe next step.

First aid and emergencies: Correct emergency pathways, recovery-position conditions and an ordered sequence.

Closing task and help routes: An individual reasoned response, a trusted-adult or safeguarding-lead route, and one next step.

Safeguarding and response

Listen calmly to any disclosure. Do not probe, investigate or promise secrecy. Follow the school safeguarding procedure and pass concerns promptly to the safeguarding lead. Use the separate safeguarding record; keep real concerns out of learning evidence. Privately support a learner who cannot identify a trusted adult.

Use the agreed pause signal. No public reason is needed. An adult supports the agreed supervised route, a neutral task and a supported return. A learner may pass, observe or catch up privately. Keep supervision in place.

VF verbal feedback · WS worked with support · I independent · NS next step in green · // self-edit · ? read this back to me · E actual evidence banked · R responded. R only after Voice and Audience are present, one NS is set and the learner responds in the same colour, on the same page or usual mode.

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